

THE SOMATIC RESET SERIES

Somatic Tools for Grief

Processing Loss Through the Body

Disclaimer

This book is intended for educational and informational purposes only. It is not a substitute for professional medical, psychological, or psychiatric advice, diagnosis, or treatment. Always consult a qualified healthcare provider before making changes to your health routine, especially if you have a pre-existing condition.

The practices described here are general wellness approaches. Results will vary between individuals. The author and publisher accept no liability for outcomes arising from use of this material.

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Grief Lives in the Body

Grief has a physical address. It lives in the chest — that actual ache that isn't metaphor. In the throat. In the heaviness of limbs that don't want to move. In the inability to eat, or the inability to stop eating. The body knows about loss in ways that precede language, and healing grief requires meeting it where it actually lives.

What Grief Does to the Nervous System

Loss activates the nervous system in ways that are physiologically significant. Research by George Bonanno and others has shown that grief produces measurable changes in cortisol levels, immune function, sleep architecture, cardiovascular activity, and inflammatory markers. These are not psychosomatic — they are direct biological responses to the disruption of an attachment bond.

The autonomic nervous system experiences grief as a safety threat. When someone we are attached to — or something we are attached to, including a role, a future, a home — is no longer present, the nervous system loses a regulatory resource. We co-regulate with people we love. Their presence, their voice, their physical warmth

help maintain our own nervous system stability. When they're gone, the system has to find new equilibrium. That process is what grief is, partly.

This is why grief can produce symptoms that look like anxiety (the searching, the hypervigilance, the scanning for the person who isn't there), symptoms that look like depression (the shutdown, the flatness, the withdrawal), and symptoms that look like physical illness (fatigue, pain, lowered immunity). It is all of these things and none of them, exactly. It is the nervous system reorganising itself around an absence.

The Problem with Grief That Gets Stuck

Grief moves. When it's allowed to, it moves through waves — intense activation followed by rest, activation followed by rest. The physiological stress response rises, peaks, and passes. Over time, the waves become less frequent and less overwhelming, and the nervous system integrates the loss.

Grief gets stuck when the waves are not allowed to complete. When crying is suppressed, when the physical activation of grief is interrupted by the need to be functional, when there is not enough safety or time to feel the feelings — the stress response activates but doesn't discharge, and the grief doesn't move. It becomes stored tension, frozen activation, chronic

low-level distress.

Somatic tools for grief are not about processing grief faster or bypassing the pain. They're about creating conditions where the physiological activation of grief can complete — so it can move, rather than become lodged.

"Grief is not a problem to solve. It's a process to support."

Creating Safety for Grief

Before any processing can happen, there has to be enough safety. This is the foundation everything else rests on.

Titration — Working in Small Doses

One of the most important concepts in somatic grief work is titration — working with grief in small, manageable doses rather than trying to feel everything at once. Full immersion in grief can be retraumatizing and doesn't actually accelerate processing. Small doses, followed by returns to a stable baseline, are how the nervous system learns to tolerate and integrate loss.

Practical titration: set a time container. Allow yourself to feel fully for twenty minutes — with whatever physical expression the grief calls for: crying, moving, making sound — then stop, do a grounding practice, and return to ordinary activity. This is not suppression. It's paced engagement.

Pendulation — Between Grief and Resource

Pendulation is a concept from Somatic Experiencing — the practice of deliberately moving attention between

the site of pain and a resource. A resource, in this context, is anything that feels genuinely supportive: a memory of the person who is gone, a physical sensation of comfort, a place in the body that doesn't hurt, a sensory experience that brings even a mild sense of okayness.

Moving attention back and forth between the grief and the resource — rather than staying immersed in the grief — allows the nervous system to process without being overwhelmed. It teaches the system that it can touch the pain and return from it.

*"You don't have to carry grief alone and you
don't have to face it all at once."*

Somatic Practices for Active Grief

These practices are for the acute phases of grief — the times when the body is activated and needs support in moving through rather than getting stuck.

Allowing the Cry

Crying is a physiological discharge mechanism. It releases stored tension, activates the parasympathetic system, and produces a specific neurochemical response — including the release of oxytocin — that produces the paradoxical sense of relief that often follows a full cry.

Many people suppress crying reflexively — particularly in public, or in the presence of others who are uncomfortable with it. Practising allowing the cry in private, with full physical expression — sound, movement, letting the face do what it wants to do — can be a genuine release mechanism for grief that has been held.

Creating a cry container: dim light, privacy, a pillow or blanket, and perhaps music that opens the channel.

Allow ten to twenty minutes. Then ground: feet on floor, several slow breaths, something warm to drink.

Movement for Grief Energy

Grief produces physical activation that needs movement. The restlessness, the inability to sit still, the urge to pace — these are the nervous system trying to discharge mobilised energy that doesn't have anywhere to go.

Walking, particularly slow walking in nature or another quiet environment, is one of the most consistent somatic grief tools because it matches the natural rhythm of grief waves — moving through them rather than trying to stop them. The bilateral movement of walking also has a gentle processing effect, similar to the bilateral stimulation used in EMDR.

Expressive movement — moving the body however it wants to move, without structure or performance — can be a powerful discharge tool for grief, particularly for people who find talking about their feelings less accessible than moving through them.

The Body Holds the Connection

One aspect of somatic grief work that is rarely discussed is the way the body holds our connection to those we've lost. Smell is particularly powerful — it bypasses the

cognitive brain and goes directly to the limbic system, producing the vivid, immediate memory response that a rational description cannot. Holding an object belonging to someone who has died, returning to a place that carries their presence — these sensory engagements are not morbid or unhealthy. They are ways the nervous system maintains connection while simultaneously processing loss.

"What the body loved, the body grieves. Give it the space to do so."

Living Forward

Grief changes over time. Not by becoming smaller, exactly, but by changing shape — and by the rest of life gradually growing larger around it.

Continuing Bonds

Modern grief research, particularly the work of Klass, Silverman, and Nickman, has challenged the older view that healthy grief requires 'letting go' and severing the bond with the person who has died. The research suggests that maintaining an ongoing, transformed relationship with the deceased — talking to them, carrying their values, feeling their presence in meaningful places — is associated with better grief outcomes, not worse ones.

Somatic practices can support this: creating a physical ritual of connection, keeping sensory anchors, returning to places that held shared meaning. These are not signs of inability to move forward. They are ways of carrying love across the boundary of loss.

When to Seek Support

Grief that is complicated — particularly when it involves traumatic loss, sudden loss, suicide bereavement, or

when it is not moving at all after many months — generally requires professional support. Complicated grief disorder is a real clinical presentation that responds well to specific therapeutic approaches, including somatic therapies.

If grief is preventing you from functioning, eating, sleeping, or finding any moments of relief after several months, please seek support. This is not a sign of weakness. It is a sign that the loss was enormous and the nervous system needs more than individual practice to process it.

"Love doesn't end when someone dies. It finds a different form. Grief is what it feels like while the transformation is happening."
